



PERSONA

Care-Plan Carla

Nursing Student Learner · Community College
ADN/AAS

*VRpatients Brand Foundation · Audit date 2026-05-22 · v2 · Locked palette
2026-05-25 · Run 4 of 6*

Confidence tags **[BEDROCK]** evidence-locked **[CRM-BACKED]** CRM-supported **[ASSUMPTION]** needs human input

1. Identity

Care-Plan Carla is a community-college nursing student drowning in care plans, terrified of NCLEX, who never signs a PO but whose course-evaluation comments and first-attempt pass rate decide whether VRpatients renews next July. **[BEDROCK]**

2. Role Type

Primary: User. Zero buying authority. Indirect influence only via course evaluations, NCLEX pass-rate contribution, attrition, and accreditation-visit interviews. **[BEDROCK]**

Product touch: VR simulation (where she lives) and the WebGL fallback (browser path when motion comfort forces it). Never touches the authoring tool. **[BEDROCK]**

Priority caveat: Carla sits in a segment that closes at ~half the EMS win rate — nursing-named deals win ~31% vs EMS ~57% (text-match proxy, N=85 vs 342). **[CRM-BACKED]** WHY owned by Director Dale Q D2.

3. Snapshot

- **Segment.** Community-college ADN/AAS pre-licensure RN candidate. **[BEDROCK]**
- **Setting.** Cohort of 20–40; shared sim lab; one or two overworked sim coordinators stage her headset session — her experience is mediated by an instructor whose bandwidth caps platform use. **[BEDROCK]**
- **Tech comfort — bimodal.** 18–22 cohort tolerates rough UI; 30–45 career-changer cohort gets nauseated faster and arrives skeptical. AACC Fall 2023: 40% of CC students are 22+, average age 27. **[ASSUMPTION]** macro stat solid; VRpatients-deployment incidence unmeasured.

- **In-life context.** 12–16 credits, 300–600 clinical hours, HESI/ATI predictors every semester, NCLEX at the end. Care plans due the night before clinical. Three hours of sleep is normal. **[BEDROCK]**
- **Tailwind.** 93,176 qualified nursing applicants turned away in 2025 (AACN). ~22 state BONs cap simulation at 50% of clinical hours. **[BEDROCK]**

4. Goals

- **Functional.** Pass every check-off, predictor, and NCLEX-RN on first attempt. **[BEDROCK]**
- **Emotional.** Not feel stupid in front of peers or instructors. Feel “ready” before a real patient. **[BEDROCK]**
- **Social.** Belong to the cohort; not be the one who fails. Justify the financial / family sacrifice. **[BEDROCK]**

5. Pains (ranked, most acute first)

1. **(Emotional/existential) Terror of looking incompetent while being graded.** The headset reshapes the fear (no one-way mirror) but does not eliminate it. **[ASSUMPTION]** transposed from general sim-lab VOC; not yet observed in VRpatients-deployed students (C3).
2. **(Functional) Care plans, concept maps, SBAR handoffs eating every weeknight.** The baseline grind. **[BEDROCK]**
3. **(Functional) First-rotation panic in peds and OB above all** — the specialty VRpatients does not yet ship. Pediatric MVP on 8–12yo avatars in progress. **[CRM-BACKED]** 4 of 96 commented lost deals cite peds; never positively.
4. **(Functional) Cybersickness** — age-correlated, hits career-changers harder. **[ASSUMPTION]** web n=38 (Germany), n=129 (TAM); deployed-account incidence unmeasured (C4).
5. **(Emotional) Graded by something she can't appeal to.** Opaque AI grader is worse than a strict human. **[ASSUMPTION]** 0 CRM mentions in 96 commented losses. Forecasted, not observed (C2).
6. **(Financial) Career-changer opportunity cost.** Lost wages + childcare. Stethoscope, UWORLD, HESI/ATI, \$200 NCLEX fee on top. **[BEDROCK]**
7. **(Social) Humiliation by a preceptor on rotation.** “Nurses eat their young.” Wants to arrive having already practiced. **[BEDROCK]**

6. What She's Tried Before & Why It Failed

Her cohort runs the same six high-fidelity manikin scenarios every semester because that's all the sim coordinator has time to build. Screen-based sim (vSim, Shadow Health, Lippincott) handles assessment-style cases but doesn't pressure-test clinical judgment the way Next Gen NCLEX wants. The manikin lab is loathed in nursing-student voice (“one-way mirrors and creepy mannequin made me feel incompetent”). **[ASSUMPTION]** manikin loathing sourced from general allnurses threads (C10).

Where peer VR has failed her: the institution bought a headset platform during COVID, the sim coordinator never had time to author, and she experienced it once before it sat in a closet. Her first-impression friction is structurally upstream of the product. **[CRM-BACKED]** “bought-but-never-used” pattern (Tasha Q5).

7. Trigger Events

- **Failed skills check-off or sim grade** — wants reps before retake. [BEDROCK]
- **Upcoming HESI/ATI predictor or final-semester NCLEX** — anxiety peak. [BEDROCK]
- **Clinical site shortage cancels a rotation** — school pushes sim; she arrives skeptical. [BEDROCK]
- **First peds, OB, or psych rotation approaching** — specialty she's never seen on a real patient. [BEDROCK]
- **Preceptor humiliated her on the floor** — wants to rehearse competence in private. [BEDROCK]
- **Instructor introduces the headset for the first time.** Trigger is institutional, not personal; her attitude is shaped by the framing. [ASSUMPTION] instructor-as-chokepoint (C7).

8. Objections

1. **"I'd rather have real clinical hours."** — Don't need it. Often emotional; sees sim as a downgrade forced on her by site shortages. [BEDROCK]
2. **"VR makes me nauseated."** — Won't work for me. Career-changers especially; "I wear glasses"; "I'm not good at video games." [ASSUMPTION] class well-documented; VRpatients-specific incidence unmeasured.
3. **"How can a computer fairly grade my therapeutic communication?"** — Don't trust the vendor. Corollary: "Can it understand my accent?" [ASSUMPTION] forecasted, not observed in CRM.

9. Watering Holes

- **Forums.** allnurses.com ("General Student Support," "Pre-Nursing Student") — most concentrated pre-licensure verbatim. [BEDROCK]
- **Reddit.** r/StudentNurse (~103K), r/nursing, r/NCLEX, r/passNclex, r/nursingschool. [BEDROCK]
- **Association.** National Student Nurses Association (NSNA) — 50,000+ members across 1,500 programs; Annual Convention every April; MidYear each fall. [BEDROCK]
- **Creators.** Nurse Sarah / RegisteredNurseRN (~1.8M YT), NurseInTheMaking (1.6M TikTok), Nurse Dani, Nurse Zoey, Nurse Meg. [ASSUMPTION] reach metric only; student-program adoption per creator unverified.
- **Test prep.** UWorld NCLEX, ATI Comprehensive Predictor, HESI Exit, Kaplan — where she spends real evening time and real money. [BEDROCK]

10. Voice of Customer

Allnurses-sourced general phrasing is not yet validated against VRpatients deployed students. Single direct in-product signal exists from Cabrillo College (faculty-channeled).

"Care plan from hell. Nursing school is kicking my ass. I bombed med-surg. Kill the patient. ATI is making me cry. Pass NCLEX or die trying."

allnurses general population · [ASSUMPTION] transposition validity open (C10)

“When student give a medication or do a procedure — there’s not details — it’s just magically done. The faculty would like to see more of the steps broken down so the student has to think through how they would get the task done.”

Cabrillo College — faculty-channeled · [CRM-BACKED] single comment in 96 commented lost deals — only direct student-UX signal in CRM (C1)

Plausible 11pm Google search: “is failing sim lab the end of my career” · “how to not look stupid in clinical tomorrow” · “nclex sata strategies” · “should I drop out of nursing school.” **[ASSUMPTION]**

11. Before / After

BEFORE VRpatients works

- Six recycled manikin scenarios a semester, one shot per skill check-off, no rehearsal before the high-stakes attempt.
- First peds patient is a real child. She walks into clinical performing for the first time. **[BEDROCK]**

AFTER VRpatients works

- Unlimited safe-to-fail reps on the specialties she’s about to face. Headset is her rehearsal stage, not her judgment day.
- Instructor sees a more prepared cohort; her course-evaluation comments stop being a renewal risk and start being a renewal lever. **[ASSUMPTION]** aspirational; depends on instructor framing, scenario freshness, peds availability, AI-grading trust.

12. Confidence Ledger

[BEDROCK]

- User-only with zero buying authority.
- Care-plan / NCLEX daily reality; cohort grind.
- Peds and OB as most-feared rotations.
- allnurses / r/StudentNurse / NSNA as watering holes.
- “Bought-but-never-used” failure mode of peer VR.

[CRM-BACKED]

- Nursing-segment win rate ~half EMS (N=85 vs 342 text-match).
- Peds named in 4 of 96 commented losses, never positively.
- One direct student-UX complaint exists (Cabrillo “magically done”).
- closed_lost_reason 3/678; closed_won_reason 6/338; hs_persona 0; hs_buying_role 0 — persona role + WHY rely on lost_reason_comments (96/678 ≈ 14%).

[ASSUMPTION]

- “Felt stupid in headset” transposed from manikin-sim literature.
- AI-grade fairness as a real student complaint.
- Cybersickness in deployed accounts.
- Career-changer skepticism rate.
- Instructor-mediation effect.
- allnurses-verbatim resonance with actual VRpatients students.
- NCLEX-pass-rate delta as a demonstrable impact.

Open Questions Routed to Team

Full traceable list in brand/personas/persona-audit-open-questions.md → “Care-Plan Carla” section. Summary by team:

- **CS.** Magically-done (C1); felt-stupid transposition (C3); cybersickness (C4); instructor-chokepoint (C7).
- **CS + Sales.** Trial-student inbound (C5); course-eval visibility (C8); VOC transposition (C10).
- **CS + Product.** AI-grade fairness (C2); specialty-anxiety beyond peds (C9).
- **CS + Product + Leadership.** NCLEX delta (C6).

Composite persona. No single named customer was used. Aggregate patterns only. Pre-questionnaire base draft — not for promotion to outputs/final-deliverables/ until the sales / CS / product / leadership questionnaire closes the [ASSUMPTION] gaps.